PEDIATRIC NURSING (THEORY) · LECTURE 9 · QUESTION BANK

CNS Congenital Anomalies

العيوب الخلقية في الجهاز العصبي — بنك الأسئلة

47	6	12
Past exam questions	NCLEX-style	Flashcards

المصادر اللي اتجمعت منها أسئلة المحاضرة دي:

ميدترم 2020 · ميدترم 2021 · ميدترم 2022 · ميدترم 2024 (ترم أول 2023-2024) · كويز 2 – 2022 · كويز 2 شعبة 3 (ملف Quiz 1 and 2) · تجميعة صور ميدترمات سابقة · أسئلة سابقة

جنب كل سؤال مصدره. لو السؤال اتكرر في كذا امتحان مكتوب كل مرة بمصدرها، وعليه علامة "تكرر". الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1 ميدترم 2020

Khalid is a 3-day-old infant admitted to the pediatric surgical ward with a myelomeningocele and hydrocephalus. While caring for Khalid the nurse should:

a. Assist with the insertion of a central line
b. Position him on the right side
c. Support with mechanical ventilator
d. Cover the sac with a moist and sterile dressing

2 ميدترم 2020

Khalid is a 3-day-old infant admitted with a myelomeningocele and hydrocephalus. The recommended position for Khalid is: تكرر في 2 مصادر

a. Prone
b. Trendelenburg's
c. Fowler's
d. Supine

3 ميدترم 2020

(Matching) Anencephaly is best described as:

a. Incomplete fusion of the vertebrae that may be signaled by an overlying tuft of hair
b. A protrusion of nerve tissue and meninges through a skull defect
c. Absence of the cerebral hemispheres
d. A sac of meninges and CSF protruding through a vertebral defect without nerve tissue

4 ميدترم 2020

(Matching) Encephalocele is best described as:

a. Incomplete fusion of the vertebrae that may be signaled by an overlying tuft of hair
b. A protrusion of nerve tissue and meninges through a skull defect
c. Absence of the cerebral hemispheres
d. A sac of meninges and CSF protruding through a vertebral defect without nerve tissue

5 (Matching) Spina bifida occulta is best described as: ميدترم 2020

a. Incomplete fusion of the vertebrae that may be signaled by an overlying tuft of hair
b. A protrusion of nerve tissue and meninges through a skull defect
c. Absence of the cerebral hemispheres
d. A sac of meninges and CSF protruding through a vertebral defect without nerve tissue

6 (Matching) Meningocele is best described as: ميدترم 2020

a. Incomplete fusion of the vertebrae that may be signaled by an overlying tuft of hair
b. A protrusion of nerve tissue and meninges through a skull defect
c. Absence of the cerebral hemispheres
d. A sac of meninges and CSF protruding through a vertebral defect without nerve tissue

7 When caring for an infant with spina bifida, which technique is most important in recognizing possible hydrocephalus? ميدترم 2021

a. Obtaining skull X-ray
b. Measuring head circumference
c. Performing a lumbar puncture
d. Magnetic resonance imaging

8 The nurse informed doctors that the WBC in CSF culture is up to 1000 and the CSF is turbid for a child with a VP shunt. What is the nurse's interpretation? ميدترم 2021

a. Normal shunt function
b. Need for repeated CSF culture
c. Infection in CSF is indicated
d. Normal WBC and color of CSF

9 The major cause of congenital hydrocephalus is brain tumor. (True / False) ميدترم 2021

a. True
b. False

10 What is the major cause of congenital hydrocephalus that interrupts ventricular circulation in the brain? ميدترم 2022

a. Meningitis
b. Head injury
c. Brain tumor
d. Aqueduct stenosis

11 The nurse informed doctors that the WBC in CSF culture is up to 1000 for a child connected to a VP shunt. What is the nurse's interpretation? ميدترم 2022

a. Indicative of CSF infection
b. Need for repeated CSF culture
c. Normal WBC of CSF
d. Normal shunt function

12 The most important preoperative nursing action for a child with myelomeningocele is: ميدترم 2024 (ترم أول 2023-2024)

a. Examination of the membranous cyst
b. Diapering and sponge bath for the child
c. Apply a sterile, moist, non-adherent dressing
d. Keep the infant in the prone position

13 Which of the following nursing actions is most important in caring for a child with hydrocephalus? ميدترم 2024 (ترم أول 2023-2024)

a. Measure vital signs
b. Assess weight
c. Monitor urinary output
d. Measure head circumference

14 One of the following is the most important nursing intervention for an infant born with spina bifida cystica: كويز 2 – 2022

a. Provide psychological support to the family
b. Assess neurologic function
c. Assess growth measurement
d. Cover the defect with a saline-soaked dressing

15 Postoperative nursing care of a child after insertion of a ventriculoperitoneal shunt is ..... كويز 2 – 2022

a. Administer narcotics for pain control
b. Check the urine for glucose and protein
c. Monitor for increased temperature
d. Assess body weight

16 When caring for an infant with spina bifida, which measure is most important in recognizing possible hydrocephalus? كويز 2 شعبة 3 (ملف Quiz 1 and 2)

a. Obtain skull X-ray
b. Measure head circumference
c. Perform a lumbar puncture
d. Magnetic resonance imaging

17 Which priority intervention should the nurse perform for the child's position after insertion of a shunt to relieve hydrocephalus? كويز 2 شعبة 3 (ملف Quiz 1 and 2)

a. Supine position
b. On the non-operative site
c. Semi-Fowler position
d. Upright position

18 Khalid is a 3-day-old infant admitted with a myelomeningocele and hydrocephalus. The recommended position for Khalid is: تكرر في 2 مصادر تجميعة صور ميدترمات سابقة

a. Fowler's
b. Trendelenburg's
c. Prone
d. Supine

19 Khalid is a 3-day-old infant admitted with a myelomeningocele and hydrocephalus. While caring for Khalid the nurse should: تجميعة صور ميدترمات سابقة

a. Assist with the insertion of a central line
b. Position him on the right side
c. Support with mechanical ventilator
d. Cover the sac with a moist and sterile dressing

20 Khalid is a 3-day-old infant admitted with a myelomeningocele and hydrocephalus. The recommended position for Khalid is: تكرر في 2 مصادر تجميعة صور ميدترمات سابقة

a. Prone
b. Trendelenburg's
c. Fowler's
d. Supine

21 **Khalid is a 3-day-old infant admitted with a myelomeningocele and hydrocephalus. The recommended position for Khalid is:** تكرر في 2 مصادر

تجميعة صور ميدترمات سابقة

a. Fowler's
b. Trendelenburg's
c. Prone
d. Supine

22 **The most common complication for VP shunt is:**

أسئلة سابقة

a. Shunt disconnection
b. Seizures
c. Fever
d. Obstruction

23 **An infant is born with a sac protruding through the spine, containing CSF, a portion of meninges and nerve roots. the condition is referred to as**

أسئلة سابقة

a. Meningocele
b. Myelomeningocele
c. Spina bifida occulta
d. Anencephaly

24 **The nurse is caring for an infant with a myelomeningocele. the parents ask the nurse why they keep measuring the babys head circumference. select the nurses best response**

أسئلة سابقة

a. Babies heads are measured to ensure growth is on track
b. Babies with a myelomeningocele are at risk for hydrocephalus, which shows up as an increase in head size
c. Because your baby has an opening on the spinal cord, your infant is at risk for meningitis, which can show up as an increase in head size
d. Many infants with myelomeningocele have microcephaly, which can show up as a decrease in head size

25 **A parent of a newborn diagnosed with myelomeningocele asks what is a common long term complication. the nurses best response is**

أسئلة سابقة

a. Learning disabilities
b. UTI
c. Hydrocephalus
d. Decubitus ulcers and skin breakdown

26 **Which would be included in the plan of care for a hospitalized newborn following surgical repair of a myelomeningocele? (Select all that apply)**

أسئلة سابقة

a. Skull X-rays
b. Daily head circumference measurements
c. MRI scan
d. Vital signs every 6 hours
e. Holding to breastfeed

27 **Which is included in the plan of care for a newborn who has myelomeningocele**

أسئلة سابقة

a. Place the child in the prone position with a sterile dry dressing over the defect. slowly begin oral gastric feeds to prevent the development of nectrozing enterocolitis
b. Place the child in the prone position with a sterile dry dressing over the defect. begin IV fluids to prevent dehydration
c. Place the child in the prone position with a sterile moist dressing over the defect. slowly begin oral gastric feeds to prevent the development of necrotizing enterocolitis
d. Place the child in the prone position with a sterile moist dressing over the defect. begin IV fluids to prevent dehydrations

28 **Which does the nurse include in the postoperative plan of care of a child with myelomeningocele following ligament release?** أسئلة سابقة

a. Encourage resuming a regular diet, beginning slowly with bland foods such as bananas
b. Encourage the child to blow balloons to increase deep breathing
c. Assist the child to change positions to avoid skin breakdown
d. Provide education on dietary requirements to prevent obesity and skin breakdown

29 **The nurse is working in a school health clinic and a teen mentions that her older sister just had a baby born with a myelomeningocele. the teen is wondering if there is anything she can do to prevent this from happening to her baby when she decides to have children. which is the best response** أسئلة سابقة

a. Take a multivtaim with folic acid daily
b. Eat more fruits and vegetables daily
c. Have breakfast every morning
d. There is nothing that can be done to decrease the risk

30 **The nurse is caring for an infant with myelomeningocele who is going to surgery later today for closure of the sac. which would be a priority nursing diagnosis before surgery** أسئلة سابقة

a. Alteration in parent infant bonding
b. Altered growth and development
c. Risk of infection
d. Risk for weight loss

31 **Which should be included in the plan of care for a newborn with myelomeningocele who will have a surgical repair tomorrow** أسئلة سابقة

a. Offer formula every 3 hours
b. Turn the infant back to front every 2 hours
c. Place a wet dressing on the sac
d. Provide pain medication every 4 hours

32 **Which should the nurse do first when caring for an infant who just had a repair of a myelomeningocele** أسئلة سابقة

a. Weight diapers for 24 hour urine output
b. Measure head circumference
c. Offer clear fluids
d. Assess for infection

33 **Which should be the priority ND for a 12 hour old newborn with a myelomeningocele at L2** أسئلة سابقة

a. Altered bowel elimination r/t neuro deficits
b. Potential for infection r/t the physical defect
c. Altered nutrition r/t neuro deficit
d. Disturbance in self concept r/t physical disability

34 أسئلة سابقة

Over the last week an infant with repiared myelomeningocele has had a high pitched cry and been irritable. length, weight and head circumference have been at the 50th percentile. today length is at the 50th percentile, weight is at the 70th percentile and head circumference is at the 90th percentile. the nurse should do which of the following

a. Tell the parent this is normal for an infant with a repaired myelomeningocele
b. Tell the parent this might mean the baby has increased icp
c. Suspect the baby's icp is low because of a leak
d. Refer the baby to the nuerologist for follow up care

35 أسئلة سابقة

Which should the nurse prepare the parents of an infant for the following surgical repair and closure of a myelomeningocele shortly after birth. the infant will:

a. Not need any long term management and should be considered cured
b. Not bo at risk for UTI or movement problems
c. Have continual drainage of CSF needed frequent dressing changes
d. Need lifelong management of urinary, orthopedic and neurological problems

36 أسئلة سابقة

A newborn with a repaired myelomeningocele is assessed for hydrocephalus. which would the nurse expect in an infant with hydrocephalus

a. Low pitched cry and depressed fontanel
b. Low pitched cry and bulging fontanel
c. Bulging fontanel and downwardly rotated eyes
d. Depressed fontanel and upwardly rotated eyes

37 أسئلة سابقة

A nurse is caring for a neonate born 12 hours ago with a myelomeningocele. Which assessment finding should be reported immediately to the PCP

a. Axillary temp of 102.2 (fever)
b. Dribbling of urine
c. No lower limb movement
d. Talipes equinavaraus

38 أسئلة سابقة

An infant has returned to the peds unit after repair of a meylomeningocele. The nruse notices that the infant has had no urine output in the past 2 hours. Which nursing intervention would be most appropriate

a. Perform credes maneuver on the infants bladder
b. Catheterize the infants bladder
c. Ask the mother to breast feed the infant
d. Increase the IV fluid rate

39 أسئلة سابقة

The parents of a child newly dx with myelomeningocele ask the nurse why surgical repair needs to be done immediately. Which reponse would be most accurate

a. Its done to rapidly restore neural pathways to the legs
b. Its done to decrease the possibility of infection and further cord damage
c. Its done to expose the spinal cord defect and allow an individualized therapeutic strategy
d. Its done to remove excess nerve tissue from the vertebral canal and to decrease pressure on the cord

40 The most important pre-operative nursing action for child with myelomeningocele is: أسئلة سابقة

a. Examine of the membranous cyst
b. Diapering and sponged path for the child
c. Apply sterile, moist, non-adherent dressing
d. Keep infant in the prone position

41 The nurse is working in the PICU caring for an infant who has just returned from having a shunt placed for hydrocephalus. Which position initially will be most beneficial for this child? أسئلة سابقة

a. Semi-Fowler in an infant seat
b. Flat in a crib
c. Trendelenberg
d. In crib with HOB elevated 90 degrees

42 Early signs and symptoms of hydrocephalus in an infant include which of the following? أسئلة سابقة

a. Confusion, headache, diplopia
b. Rapid head growth, poor feeding, confusion
c. Papilledema, irritability, headache
d. Full fontanels, poor feeding, rapid head growth

43 Which of the following nursing action is most important in caring of child with hydrocephalus? أسئلة سابقة

a. Measure vital signs
b. Assess weight
c. Monitor urinary output
d. Measure head circumference

44 The nurse informed doctors about the WBC result in CSF culture up to 1000 for child connected with VP shunt, what the nurse interpretation of this result أسئلة سابقة

a. Indicted of CSF infection
b. Need for repeated CSF culture
c. Normal WBC of CSF
d. Normal shunt function

45 Khalid is 3 days old infant admitted to the pediatric surgical ward with a myelomeningocele and hydrocephalus while caring for Khalid the nurse should أسئلة سابقة

a. Assist with the insertion of a central line
b. Position him on the right side
c. Support with mechanical ventilator
d. Cover sac with a moist and sterile dressing

46 Khalid is 3 days old infant admitted to the pediatric surgical ward with a myelomeningocele and hydrocephalus the recommended position for Khalid is أسئلة سابقة

a. Prone
b. Trendelburg's
c. Fowler's
d. Supine

47 What is the major cause for congenital hydrocephalus that interrupted ventricle circulation in the brain? أسئلة سابقة

a. Meningitis
b. Head injury
c. Brain tumor
d. Aqueduct stenosis

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

N1 **The priority preoperative care for a newborn with myelomeningocele is:** NCLEX

a. Supine positioning
b. Covering the sac with a sterile, moist, non-adherent dressing
c. Applying a dry pressure dressing
d. Allowing the parents to hold the baby upright

N2 **Which is an early sign of hydrocephalus in an infant?** NCLEX

a. Headache and diplopia
b. Rapidly increasing head circumference with bulging fontanel
c. Papilledema
d. Decreased head circumference

N3 **Select ALL signs of VP shunt malfunction or infection.** SELECT ALL

a. Vomiting and irritability
b. Bulging fontanel
c. Fever
d. Increased appetite
e. High-pitched cry

N4 **Which advice helps prevent neural tube defects?** NCLEX

a. Avoid vitamin supplements
b. Folic acid 0.4 mg daily before conception and in early pregnancy
c. High-protein diet in the 3rd trimester
d. Iron supplements only

N5 **Spina bifida occulta is often signaled by:** NCLEX

a. A large sac on the back
b. A tuft of hair or dimple over the lumbosacral spine
c. Absent cerebral hemispheres
d. Open skull defect

N6 **Immediately after VP shunt insertion, the infant is usually positioned:** NCLEX

a. On the operative side
b. Flat, on the non-operative side
c. Head down
d. Sitting upright

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Anencephaly	Absence of cerebral hemispheres; incompatible with life.
Encephalocele	Protrusion of brain tissue and meninges through a skull defect.
Spina bifida occulta	Hidden vertebral defect; tuft of hair or dimple.
Meningocele	Sac with meninges and CSF only; no nerve tissue.

Myelomeningocele	Sac with meninges, CSF and spinal cord/nerve roots.
MMC pre-op	Prone, sterile moist non-adherent dressing, protect from feces/urine, measure head circumference.
MMC complications	Hydrocephalus, UTI, neurogenic bladder, paralysis.
Hydrocephalus cause	Commonly aqueduct stenosis (obstructive).
Hydrocephalus signs	Large head, bulging fontanel, sunset eyes, high-pitched cry, irritability.
Most important hydrocephalus assessment	Daily head circumference.
VP shunt complications	Obstruction (most common) and infection.
Prevention	Folic acid before conception and in early pregnancy.

Answer Key

الإجابات

1. d — Cover the sac with a moist and sterile dressing

2. a — Prone

3. c — Absence of the cerebral hemispheres
وصف الـ) سؤال توصيل في الامتحان الأصلي، اتحوّل لاختيار من متعدد. meningocele مكتوب من الملخص لأنه كان مقصوص في الصورة).

4. b — A protrusion of nerve tissue and meninges through a skull defect
وصف الـ) سؤال توصيل في الامتحان الأصلي، اتحوّل لاختيار من متعدد. meningocele مكتوب من الملخص لأنه كان مقصوص في الصورة).

5. a — Incomplete fusion of the vertebrae that may be signaled by an overlying tuft of hair
وصف الـ) سؤال توصيل في الامتحان الأصلي، اتحوّل لاختيار من متعدد. meningocele مكتوب من الملخص لأنه كان مقصوص في الصورة).

6. d — A sac of meninges and CSF protruding through a vertebral defect without nerve tissue
وصف الـ) سؤال توصيل في الامتحان الأصلي، اتحوّل لاختيار من متعدد. meningocele مكتوب من الملخص لأنه كان مقصوص في الصورة).

7. b — Measuring head circumference

8. c — Infection in CSF is indicated

9. b — False

10. d — Aqueduct stenosis

11. a — Indicative of CSF infection

12. c — Apply a sterile, moist, non-adherent dressing
الإجابة الصح حسب بنك الأسئلة — (a) ورقة الطالب معلّم عليها والملخص: الغيار المعقّم المبلول غير اللاصق.

13. d — Measure head circumference

14. d — Cover the defect with a saline-soaked dressing

15. c — Monitor for increased temperature

16. b — Measure head circumference

17. b — On the non-operative site

18. c — Prone

19. d — Cover the sac with a moist and sterile dressing

20. a — Prone

21. c — Prone

22. d — Obstruction

23. b — Myelomeningocele

24. b — Babies with a myelomeningocele are at risk for hydrocephalus, which shows up as an increase in head size

25. b — UTI

26. b, c — Daily head circumference measurements; MRI scan

27. d — Place the child in the prone position with a sterile moist dressing over the defect. begin IV fluids to prevent dehydrations

28. c — Assist the child to change positions to avoid skin breakdown

29. a — Take a multivtaim with folic acid daily

30. c — Risk of infection

31. c — Place a wet dressing on the sac

32. b — Measure head circumference

33. b — Potential for infection r/t the physical defect

34. b — Tell the parent this might mean the baby has increased icp

35. d — Need lifelong management of urinary, orthopedic and neurological problems

36. c — Bulging fontanel and downwardly rotated eyes

37. a — Axillary temp of 102.2 (fever)

38. b — Catheterize the infants bladder

39. b — Its done to decrease the possibility of infection and further cord damage

40. c — Apply sterile, moist, non-adherent dressing

41. b — Flat in a crib

42. d — Full fontanels, poor feeding, rapid head growth

43. d — Measure head circumference

44. a — Indicted of CSF infection

45. d — Cover sac with a moist and sterile dressing

46. a — Prone

47. d — Aqueduct stenosis

N1. b — Covering the sac with a sterile, moist, non-adherent dressing
A moist sterile dressing prevents drying, rupture and infection of the sac; position prone.

N2. b — Rapidly increasing head circumference with bulging fontanel
Open sutures allow the head to enlarge; bulging fontanel, sunset eyes and irritability follow.

N3. a, b, c, e — Vomiting and irritability; Bulging fontanel; Fever; High-pitched cry
Shunt obstruction raises ICP (vomiting, bulging fontanel, high-pitched cry); fever and redness suggest infection.

N4. b — Folic acid 0.4 mg daily before conception and in early pregnancy
Periconceptional folic acid markedly reduces neural tube defects.

N5. b — A tuft of hair or dimple over the lumbosacral spine
The defect is hidden; skin signs such as a hair tuft, dimple or hemangioma may be present.

N6. b — Flat, on the non-operative side
Lying flat on the non-operative side avoids pressure on the valve and prevents too-rapid CSF drainage.